Studying Changes in Intimate Partner Violence in Latin America:

A Brief Report from a Panel Survey during COVID-19

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Abstract

*Purpose***:** Intimate partner violence (IPV) is a significant public health problem in Latin America. However, understanding the scope and dynamics of IPV has been limited by reliance on administrative or cross-sectional data. This limitation became especially problematic during the COVID-19 pandemic, when mobility restrictions likely affected help-seeking and reporting. Drawing on a new panel database, this brief report examines changes in IPV during the COVID-19 pandemic in Chile and describes features of the data and challenges associated with collection under emergency conditions.

Methods: From 2021 to 2022, we collected the second wave of a national survey focused on violence against women. The final sample includes over 3,000 women. We describe the sample and assess changes in IPV over time, including transitions across forms of violence and reporting to authorities.

Results: While victimization is correlated across waves, we observe substantial heterogeneity in IPV dynamics, with many women transitioning out of violent relationships or moving between different forms of violence. Notably, a majority of women reporting IPV in 2022 had not reported IPV in 2020, suggesting onset during the period. We also find a marked decline in formal reporting.

Conclusions: Despite relatively stable aggregate trends, IPV during the pandemic in Chile showed significant heterogeneity in individual-level transitions and a sharp decline in formal reporting. These findings illustrate how aggregate patterns can mask shifts in women's experiences over time and underscore the value of representative longitudinal survey data for capturing dynamics in victimization and reporting that administrative records and repeated cross-sections may miss.

Keywords: Intimate Partner Violence; COVID-19; Latin America; Longitudinal Survey; Chile

Introduction

Nearly one in every three women reports having been a victim of intimate partner violence (IPV) at some point during their lives (Garcia-Moreno et al., 2006). The risk of experiencing IPV varies significantly across countries, with low-income countries presenting a relatively higher prevalence of violence compared to high-income ones (Heise & Kotsadarn, 2015; White et al., 2023). In Latin America, life prevalence of sexual or physical IPV ranges between 15 to almost 60% depending on the country (Bott et al., 2021), with an intimate-partner femicide rate of 1.6, the second highest after Africa (UNODC, 2024).

The COVID-19 pandemic generated a global concern about a potential increase in domestic violence. Mandatory lockdowns and fear of contagion significantly reduced physical mobility, reshaping the risk of domestic violence by increasing the time exposed to potential perpetrators (Stickle & Felson, 2020). Many of the known risk factors associated with IPV were also exacerbated during this time, including increases in unemployment, emotional stress, and alcohol use, as well as a decrease in social support (Bhalotra et al., 2024; Brown et al., 2020; Hernandez et al., 2024). As a result, there has been a substantial wave of research focused on pandemic-driven changes in domestic violence. While initial studies tended to report increases consistent with expectations (for a review, see Bourgault et al., 2021; McNeil et al., 2023), after five years, the literature suggests significant heterogeneity across places and subpopulations (Erten et al., 2022; Piquero et al., 2021; Richards et al., 2021), and largely mixed results depending on the data sources used (Kim & Royle, 2024; Miller et al. 2024). Increases were more likely to be reported in studies that relied on calls to hotlines and during the first weeks of the pandemic (e.g., Bullinger et al., 2020; Leslie & Wilson, 2020), while reports to the police largely decreased during this time (Bullinger et al., 2020; Miller et al., 2020; but see Ravindran & Shah, 2020).

As most of these studies rely on administrative data—either reports or calls to the police—it becomes difficult to disentangle changes in prevalence from changes in reporting of domestic violence. Prior research suggests that about 30% of cases of IPV are reported to the police (Breiding et al., 2014; Voce & Boxall, 2018), a percentage that could have further decreased given the mobility restrictions implemented during COVID (Blofield et al., 2022). Victimization surveys play a key role in our understanding of domestic violence, as they do not depend on the decision to formally report and can measure characteristics of the

relationships absent in official data. The limited number of studies that use self-reported information during COVID-19, however, were based on small, non-representative, and cross-sectional surveys (e.g., Arenas-Arroyo et al., 2021; Gibbons et al., 2021).

This brief report aims to contribute to the study of intimate partner violence during COVID-19 by describing changes in self-reported victimization and examining the dynamic patterns across different types of violence during this time. We use a new longitudinal dataset that surveyed women in Chile prior to and during the COVID-19 pandemic. Unlike the majority of existing studies on IPV and COVID, the *Encuesta Nacional de Experiencias Familiares y Laborales durante la Crisis de COVID-19* (National Survey on Family and Work Experiences During the COVID-19 Crisis, ENVIF-COVID) dataset offers a rare opportunity to explore IPV within the context of a global health crisis and to analyze changes over time in the prevalence and type of violence and relationship conflict. Given the novelty of the data, we discuss the survey design, its benefits and limitations, and some of the challenges associated with data collection during the pandemic. We hope that our analyses and the availability of this data will contribute to understanding how IPV evolves in response to shifting social and economic conditions in an understudied context—Latin America.

Methods

Our study is set in Chile, a country in which, despite the notable policy progress made in addressing and responding to IPV,[1] life and yearly prevalences of violence have remained relatively stable over the last decade (Banco Mundial, 2021). In 2020, 1 in 4 women reported experiencing some type of IPV in the prior 12 months, with 26% reporting more than one type of violence (Banco Mundial, 2021).

Survey Design

The Chilean government has conducted cross-sectional surveys on domestic violence against women at irregular time frames since 2008. The National Survey on Violence Against Women (ENVCM) is representative of women aged 15 and older living in urban areas in Chile.[2] Participants are selected using a

[1] Some examples of these policy advances are the Intrafamily Violence Law Nº19325 enacted in 1994 and later amended in 2005 through Law Nº20066, the criminalization of femicide in 2010, and the establishment of the Ministry of Women and Gender Equity in 2015 (Franceschet, 2010; Banco Mundial, 2021; Saavedra et al., 2022).
[2] The survey was originally titled the National Survey on Victimization by Domestic Violence and Sexual Offenses (Encuesta Nacional de Victimización por Violencia Intrafamiliar y Delitos Sexuales). It was later renamed the National Survey on Violence Against Women (Encuesta Nacional de Violencia Contra las Mujeres) when questions about victimization outside the household were introduced.

cluster sampling method of neighborhoods and households, ensuring national and regional-level representation. In addition to sociodemographic information, the survey includes a series of questions about experiences of violence in the domestic space--distinguishing between psychological, physical, and sexual violence and violence that occurs in public spaces, workplaces, and educational settings. In the survey, domestic violence encompasses violence committed by partners, former partners, and other family members. It also includes questions about reporting incidents of violence and the consequences of such violence.

Taking advantage of the fact that the most recent survey was conducted just months before the onset of COVID-19 and aiming to contribute to the discussion on changes in IPV as a consequence of the pandemic, we conducted a follow-up survey of women who had initially participated in the 2020 National Survey on Violence Against Women (ENVCM).[3] Ethical approval was granted by the Ethical Committee of the Facultad de Economía y Negocios, Universidad de Chile (January 2021).

The sample of interest was all women who participated in the 2020 ENVCM and who had any contact information recorded (address, phone, or email).[4] For funding limitations, we initially restricted the sample to those between ages 18 and 55, then expanded to include any women ages 18 to 67 by 2021. The data were collected between September 2021 and February 2022 by trained interviewers who followed the same procedures used in 2020, guaranteeing the safety and confidentiality of the women participating in the survey. The final sample corresponds to 3,042 women, representing 52% of the population we aimed to survey.

In terms of the information collected, we restricted the survey to IPV, with questions about violence focusing solely on the partner or ex-partner. We additionally incorporate questions specific to the pandemic experience, such as fear of contagion, unemployment, access to benefits, and changes in household composition. Finally, we included life history calendars to collect information about relevant events and the timing of their occurrence.[5]

[3] This process required coordination with the Ministry of Women and Gender Equity and the Undersecretariat for Crime Prevention, the agencies responsible for the original survey. Due to data protection requirements, the data collection was carried out by the same consulting firm previously contracted by the government for the 2020 ENVCM.

[4] The sample of women in the 2020 survey within the age target was 6,775. Of these, 919 women did not have contact information available; therefore, the sample available for the follow-up survey was 5,856 women.

[5] Life calendars are well-used tools in social sciences, designed to collect detailed retrospective data and help with the recall and organization of specific events (Axinn et al., 1999; Hayes, 2018; Roberts & Horney, 2009). Its use can be particularly beneficial in IPV research as a way to measure repeated victimization (Hayes, 2018).

Therefore, the ENVIF-COVID survey consists of a two-wave longitudinal study designed to measure experiences of intimate partner violence, with Wave 1 corresponding to the 2020 survey and Wave 2 to the 2022 follow-up. While we refer to both waves as 2020 and 2022, for the year in which the data collection ended, the violence reported largely reflects the years 2018-2019 and 2020-2021.[6] To our knowledge, this is the first longitudinal survey on IPV conducted in Latin America.[7] We have made this survey available to all researchers. The codebook and dataset can be found at Harvard Dataverse (Bhalotra et al., 2024).

Challenges with data collection

The original survey (ENVCM 2020) was designed to be administered in person at the individual's place of residence. This is done to ensure that participants feel comfortable answering the questions asked and to guarantee their safety. For example, the interviewers were trained to handle interruptions, such as a partner entering the room, by smoothly changing the topic and asking a different set of questions. Because of these issues, it was crucial that this new ENVIF-COVID wave was administered in the same manner. However, this decision brought additional challenges associated with the pandemic overall and the policies adopted in Chile in particular—e.g., mandatory and very restricted lockdowns and curfews—that remained in place for a longer period compared to other countries in the region.[8] For example, although data collection was originally scheduled to begin in March 2021, it was delayed until September 2021 due to the implementation of new quarantines and because some areas remained under ongoing quarantine restrictions. Thus, participants had to recall instances of IPV for a longer period of time, with additional challenges for building comparative estimates between waves.[9] In addition, the pandemic increased the likelihood of individuals changing their place of residence (Haslag & Weagley, 2024; Lei & Liu, 2023), making it more challenging to locate them. Finally, the firm that we partnered with to do the fieldwork reported difficulties in finding enumerators willing to interview individuals in their households, precisely because of the fear of contagion. Overall, the issues we

[6] The 2020 survey was collected between November 2019 and March 2020, while the 2022 wave was collected between September 2021 and February 2022.

[7] The exception is Treves-Kagan et al. (2022), analyzing neighborhood-level indicators of domestic violence in a specific area in the northern side of Ecuador. The survey was originally designed to test a cash, voucher, and food transfer intervention, with two waves collected at 6-month intervals in 2011.

[8] For additional discussion of Chile's pandemic response, see Bhalotra et al., 2024; Larroulet et al., 2025; Mena et al., 2021.

[9] Given that the time frame used for the questions regarding IPV was different in each wave (last twelve months for 2020; month-by-month incidence using the life calendar for 2022), we considered the incidence of violence in the last twelve months prior to the 2022 follow-up to construct comparable indicators of IPV prevalence.

experienced align with other studies of sensitive issues implemented during the COVID-19 pandemic (Jessell et al., 2022; Ravi et al., 2024).

In response to these challenges, we adopted a variety of strategies to increase participation. We increased the number of follow-up visits the survey team conducted to five when no one was found at the place of residence. The team strictly adhered to COVID-19 social distancing measures, offering women the option to be interviewed outside if they preferred. We extended the data collection until February 2022 in order to achieve a larger number of cases. Finally, we added an economic incentive in the form of a raffle for 100 gift cards with a value of 10,000 Chilean pesos (approximately 12 USD at the time of the survey). Owing to these efforts, the response rate for the follow-up was finalized at 52%. The three main reasons for nonparticipation were: no one present at the residence (20%), refusal to participate (15%), and locating the household without being able to determine the whereabouts of the original participant (10%).

Representativeness

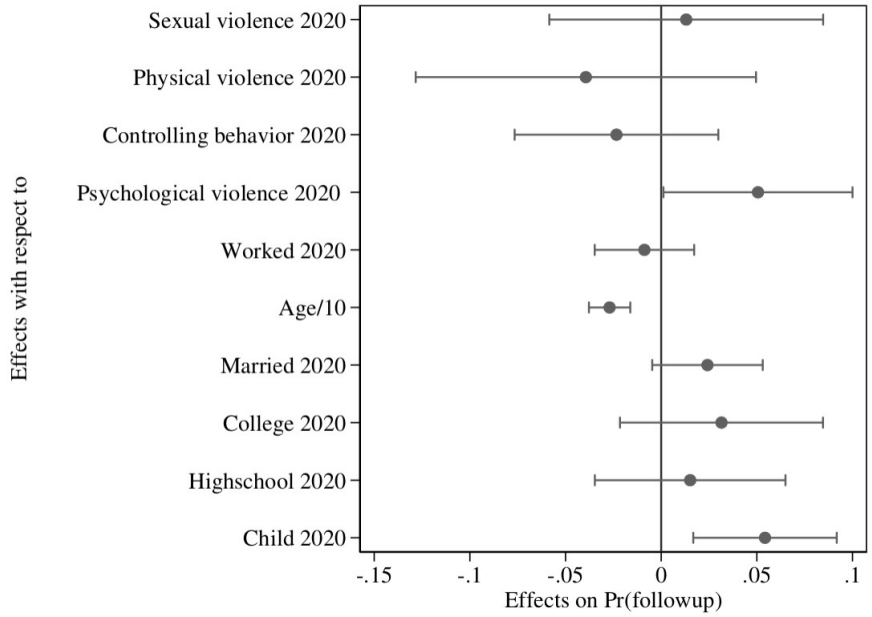
The first wave of the survey used a stratified sampling design to achieve regional and national representation of women aged 15–65, and as stated, fifty-two percent of the original sample participated in the follow-up. While the survey firm re-weighted observations based on region and age to maintain representativeness,[10] nonparticipation in the follow-up may have been driven by non-random factors.

Figure 1 presents the marginal effects of a logit regression where the dependent variable is a dummy, taking the value of 1 if the woman participated in the follow-up survey. We include as covariates socioeconomic characteristics (age, marital status, education, motherhood, and labor market participation) as well as reports of violence in the 2020 wave. Our findings show that having experienced sexual or physical violence or being in a relationship with controlling behavior during 2020 (i.e., reported at the first wave) does not decrease the likelihood of answering the survey. Regarding psychological violence, our results show that this significantly increases the likelihood of participating in the follow-up. Regarding socioeconomic characteristics, working, educational, and marital status do not have an effect. Having at least one child at baseline (2020) increases the likelihood of answering. Finally, age is the only characteristic that reduces the likelihood of participating in the survey. We hypothesize that older women may have been more worried about

[10] Those weights–by age and region– are available in the published dataset.

the in-person contact and might thus refuse to receive the elicitor. The effect, however, is modest: an increase of 10 years of age reduces the likelihood of participating in the follow-up by 2.7 p.p. Overall, the results suggest that attrition was not impacted by prior victimization, nor by some of the key factors associated with domestic violence, providing support for the quality of the data collected.[11]

Figure 1: Impact of socioeconomic characteristics and incidence of violence in 2020 on the probability of answering the 2022 follow-up survey.



***Note**:* The figure shows average marginal effects and 95% confidence intervals of a logit model where the dependent variable is a dummy for answering the follow-up survey. The regression also includes dummies for regions.

Basic Sociodemographic Descriptives of the Survey

Table 1 presents descriptive statistics for demographic and socioeconomic indicators, relationship status and fertility, and incidence of domestic violence for our sample of 3,042 women surveyed in the years 2020 and 2022. In 2022, the average age in our sample was 40.6 years. 18.5% had no schooling or incomplete high school education, 38.4% completed high school, 26.5% had post-secondary or incomplete college education, and 16.5% had college or a higher degree. Degree completion is slightly higher in 2022 compared to 2020, consistent with some women obtaining a high school diploma or a college degree in the time between surveys, while the employment rate decreased in line with the documented labor market effects of the pandemic. 47.1%

[11] Results are similar when we include the sample of women without contact information, with the exception of Married 2020, which becomes statistically significant, and Child 2020, which is larger in magnitude. This suggests that single and childless women were less likely to provide contact information to elicitors.

of women were working for pay, slightly less than in 2020. In 2022, 62% of women surveyed had a partner and 84.5% of those were cohabitating. Having a partner was slightly more common in 2020, with 66.4% of women reporting having a partner. Lastly, 75.9% of women had at least one child by 2022, with an average number of children for the whole sample equal to 1.8. We see a small increase in overall fertility between surveys, consistent with some births occurring during this period.

Table 1. Descriptive Statistics

Variable	Wave 1 2020	Wave 2 2022
Panel A: Socioeconomic and Demographic Characteristics		
Age	38.7	40.6
Education		
No schooling / Incomplete high school	24.7%	18.5%
Completed high school	33.7%	38.4%
Post-secondary / Incomplete college	26.6%	26.5%
College degree or higher	14.6%	16.5%
Working for pay	49.5%	47.1%
Panel B: Relationship Status and Fertility		
Has a partner	66.4%	62.0%
Cohabiting (among those with a partner)	82.1%	84.5%
Has children	75.0%	75.9%
Number of children	1.7	1.8
Panel C: Lifetime Experience of Domestic Violence		
Psychological violence	40.9%	–
Physical violence	16.7%	–
Sexual violence	10.7%	–
Any violence (psychological, physical, or sexual)	44.7%	–

Note: The table presents descriptive statistics obtained by weighting the sample to render it nationally representative by geographic region and age group.

Regarding lifetime experiences of domestic violence, in the 2020 survey, 40.9% of women reported having experienced psychological violence by either a family member, a partner, or an ex-partner at some point in their lives; 16.7% report having experienced physical violence, and 10.7% report having experienced sexual violence. Considering all three types of violence, 44.7% of women have experienced violence from a family member, partner, or ex-partner during their lifetime.[12]

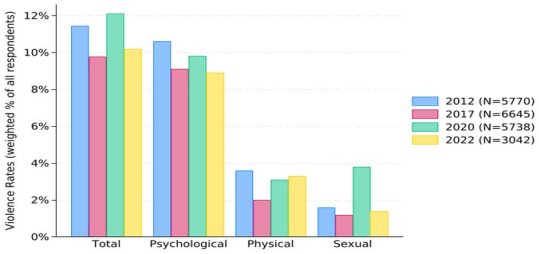
[12] Questions about incidents of psychological and physical violence before the reference period–12 months in the 2020 survey–do not distinguish between a family member, or a partner or ex-partner as perpetrators. Incidents of

Results

In what follows, we quantify changes in violence prior to and during the COVID-19 pandemic. We do so while also discussing the value added of conducting a panel survey on IPV, which is not immediately evident in cross-sectional studies, even when repeated measures are collected.

We start by analyzing the series of repeated cross-sections, consisting of the three prior waves of this survey, as well as the follow-up wave. These were conducted in 2012, 2017, 2020, and 2022, allowing us to observe the national dynamics of IPV over time. Figure 2 shows changes in the percentages of different types of violence (physical, psychological, and sexual) over the years 2012, 2017, 2020, and 2022. All indicators refer to the yearly prevalence of intimate partner violence and, therefore, reflect prevalence in the twelve months prior to the data collection. Psychological violence stands out as the most prevalent type in all years, peaking in 2017 and slightly decreasing by 2022. In contrast, physical violence exhibits lower and relatively stable values, with no significant changes over the analyzed period. Meanwhile, sexual violence, although less frequent, shows an increase by 2020, followed by stabilization in 2022. Considering the total percentage of IPV (combining the three types), a sustained increase is observed between 2012 and 2020, maintaining a relatively high level in 2022. These figures reflect differentiated patterns among types of violence, emphasizing a persistence in psychological violence as the most common form, while the other types of violence show more contained dynamics.

Figure 2: Rates of intimate partner violence over time



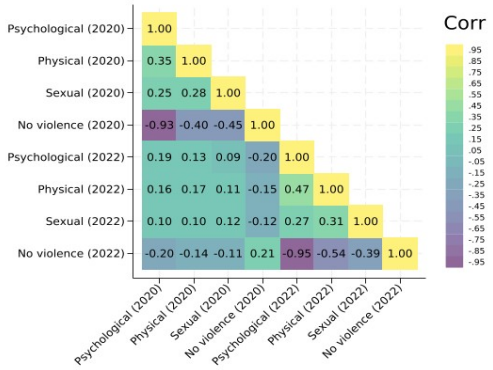
***Note**:* Rates are calculated from responses to three separate rounds of the survey (2012, 2017, and 2020), as well as the fourth panel round (2022). Each bar reports the weighted proportion of respondents indicating that they were exposed to a specific time of violence in the 12 months prior to the survey, with weights based on survey sampling weights.

While aggregate rates of IPV appeared stable, these results offer limited insight into the persistence or dynamics of violence within individuals over time, masking significant heterogeneity in IPV patterns at the

sexual violence are asked specifically as those perpetrated by a partner or ex-partner.

individual level. Figure 3 leverages the panel component of the survey (2020–2022) to examine how different types of violence co-occur within individuals and evolve over time. Unlike cross-sectional snapshots, panel data allow us to observe transitions—whether individuals remain exposed to the same type of violence, switch to a different type, or cease experiencing violence altogether. The figure highlights strong correlations within the top left and bottom right quadrants, indicating a high correlation in exposure to distinct types of violence at a given moment of time (e.g., a correlation of 0.35 between physical and psychological violence in 2020). However, what is new—and would have been obscured in cross-sectional surveys—are patterns displayed on the lower-left quadrant of the matrix, noting correlations in violence over time at the individual level. Here, we observe evidence of substantial cross-temporal correlation in victimization, such as between physical violence in 2020 and 2022 (a correlation of 0.17) or between psychological violence in 2020 and physical violence in 2022 (a correlation of 0.16), suggesting that individuals may move across types of violence over time. These patterns point to important heterogeneity in how violence unfolds across individuals, which is obscured in aggregate cross-sectional estimates.

Figure 3: Interactions between violence within individuals and across time



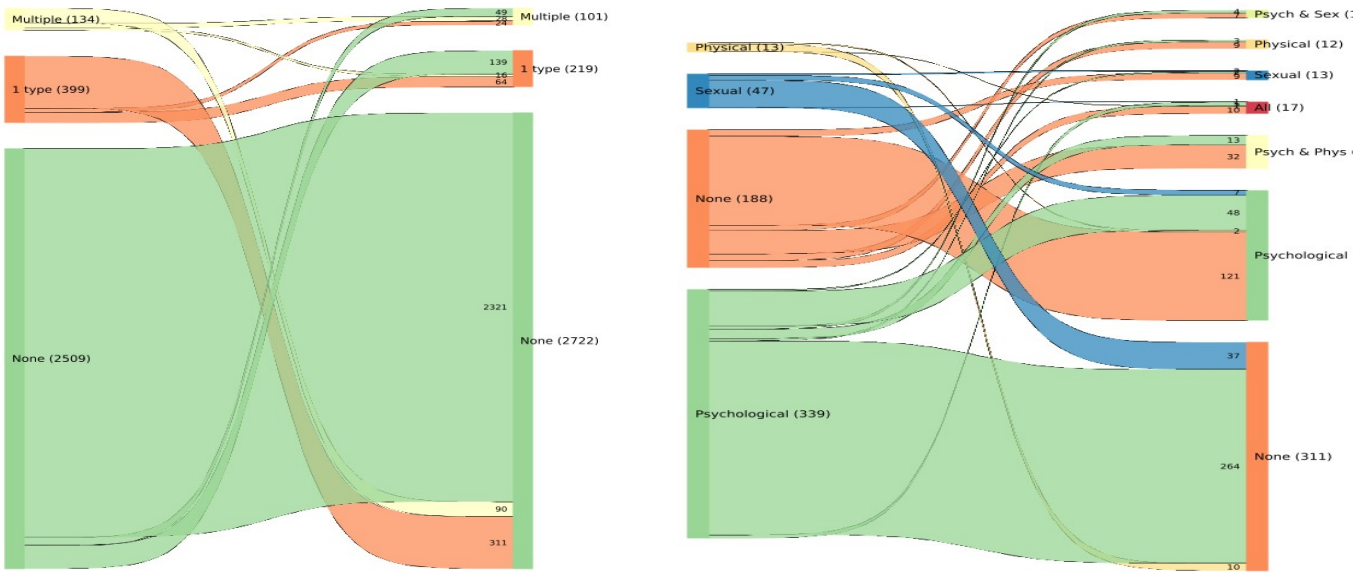
Note: "Corr" refers to simple two-way correlations between each measure. The sample of 3,042 respondents is used to calculate each correlation coefficient.

While correlations such as those documented in Figure 3 offer insights into the co-occurrence and persistence of violence types across time in the sample, the panel nature of the data allows us to trace individual-level transitions across broad categories of IPV exposure. Figure 4-Panel A illustrates these transitions between 2020 (left side) and 2022 (right side) using a Sankey diagram, grouping individuals into three mutually exclusive classes: None (individuals who reported no exposure to IPV), One type (individuals who reported having experienced only one type of violence–either physical, psychological or sexual), and

Multiple types of violence, representing women exposed to poly-victimization (Goodlin & Dunn, 2010; Ogle et al., 2024). Several important dynamics emerge. First, a large majority of respondents reporting no violence in 2020 remained violence-free in 2022 (2,321 out of 2,509, 93%), indicating stability for the largest group. However, non-negligible flows into violence are also observed: 139 individuals transitioned from no violence to one type, and 49 into multiple types by 2022. Second, individuals initially reporting one type of violence in 2020 exhibited diverse trajectories—while 16% continued in the same category, 6% escalated into multiple forms, and 311 (78%) did not experience IPV. Finally, among those experiencing multiple types of violence in 2020, about a fifth (28 out of 134) remained in that state, while others either reduced to one type or ceased reporting violence.

Figure 4: Individual-level Dynamics in IPV over time

Panel A **Panel B**



Notes: Panel A represents transitions in the exposure to violence between 2020 (left-hand side) and 2022 (right-hand side). Multiple refers to individuals reporting having been a victim of various types of violence (physical, psychological, or sexual), while 1 type refers to individuals reporting being a victim of only one of these classes. Panel B depicts transitions in the type of violence experienced between 2020 (left-hand side) and 2022 (right-hand side). Observations in Panel B are limited to individuals who report suffering either zero or one type of IPV in Wave 1 (2020) and remove the 2,321 women who report not suffering violence of any type in both waves.

We can zoom in further to examine individual-level transitions across specific types of IPV. Figure 4-Panel B captures how women's reported experiences with psychological, physical, and sexual violence evolved between 2020 and 2022. Because women can suffer multiple types of violence, we focus on the left on

women who, in 2020, experienced either a single type of violence or were not exposed to violence. For ease of visualization, we also remove the 2,321 women who reported suffering neither type of violence in 2020 and 2022. This visualization enables a fine-grained analysis of both persistence and changes in violence type, as well as transitions into poly-victimization. Given that we focus on individuals exposed to either 0 or 1 type of violence in 2020, it allows us to easily visualize the proportion of the population for which violence increases (either in magnitude or in type) over time, as well as the proportion for which violence exposure stays the same or declines.

Several patterns emerge. First, while women's exposure to violence at times intensifies (e.g., moving from psychological to psychological and sexual, psychological and physical, or all types of violence), this is relatively infrequent in this setting. In fact, the most common outcome for individuals exposed to each type of violence in Wave 1 was that they would not be exposed to violence in Wave 2. Among individuals who suffered psychological, sexual, and physical violence, the proportions of individuals who later did not report violence were 78%, 79%, and 77%, respectively. What is more, we observe that most people who were exposed to each type of violence in 2022, including all types of violence together, were women who, in the prior wave, did not experience violence at all. This result provides a new lens to the discussion about risks of onset of domestic violence during the pandemic (see Leslie & Wilson, 2020, for a related discussion).

Overall, the results illustrate that violence against women is not static: individuals move across different states of exposure, with psychological violence often persisting or replacing other forms. These transition patterns indicate both persistence and change in IPV experiences over time, underscoring the heterogeneity concealed within aggregate rates—which, if examined alone, could be misinterpreted as signaling a full cessation of harm. Together, these findings highlight the importance of identifying the risk and protective factors that shape these different pathways.

While our focus to this point has been on the experience of violence, similar lessons can be drawn related to the reporting of violence. Over time, we observe a considerable decline in the likelihood of reporting, potentially aggravated by the pandemic. In 2020, 19.4% of respondents who reported being exposed to violence reported they made a formal complaint to authorities (typically the police), while in 2022, this figure had fallen to only 10.9%. Nevertheless, when considering the dynamics of these measures over time, it

appears that these complaints do not necessarily lead to resolutions, as among individuals who reported having made a complaint with authorities in either 2020 or 2022, around 25% report making complaints in both 2020 and 2022, while the remaining 75% reported only making a complaint in one of these waves. This decrease in reporting is particularly relevant as it may help explain some of the conflicting results on changes in violence during COVID (e.g., Bhalotra et al., 2024; Miller et al., 2024; Perez-Vicent & Carrera, 2022).

Discussion

Using a new panel database collected in Chile, this brief report examines changes in intimate partner violence during the COVID-19 pandemic. The panel design offers a unique opportunity to examine not only changes in prevalence but also the persistence and transformation of violence during this time, and to assess the generalizability of current findings in a different and under-studied context. In contrast to most existing studies on IPV and COVID that rely heavily on administrative records, our data offer valuable insights into the dynamic nature of violence and reporting within the broader social and economic disruption of a global crisis.

A first central finding is that overall rates of IPV remained relatively stable at the aggregate level between 2020 and 2022, with a slight decrease in psychological and sexual violence in the 2022 survey. This finding is notable given the context of COVID-19, which intensifies many known risk factors for IPV, including financial stress, unemployment, and social isolation (Bhalotra et al., 2021; Carney, 2024; Stickle & Felson, 2020). Although many expected violence to increase, existing evidence is mixed (Kim & Royle, 2024), and self-report studies have often relied on smaller, non-representative, cross-sectional samples (e.g., Arenas-Arroyo et al., 2021; Gibbons et al., 2021). Our longitudinal data from a general population sample suggest that, contrary to expectations, the most common trajectory over this period was toward non-violence.

At the same time, the panel structure reveals substantial individual-level heterogeneity beneath stable aggregate trends, underscoring why longitudinal designs are especially informative for understanding IPV (e.g., Bassuk et al., 2006; Giordano et al., 2015). Many women exited violent relationships, while others transitioned from non-exposure into IPV or from one form of violence into multiple types over time. Such transitions are obscured in cross-sectional prevalence estimates and are particularly relevant for understanding poly-victimization—whether defined as co-occurring types of victimization within a period or the experience of different forms across time and partners (Goodlin & Dunn, 2010). Our data show that women often

experience different types of violence over time, and that transitions into poly-victimization are common. Taken together, these findings highlight that stability in prevalence can conceal substantial churn in individual experiences and shifts in the composition of violence across time.

The observed heterogeneity also raises new questions about the experience of violence during COVID, particularly whether and for whom the pandemic led to changes in participation versus changes in intensity of domestic violence. For example, what facilitated transitions for the large number of women who ceased experiencing IPV? Were changes in partners, household composition, or living arrangements associated with reductions in victimization? Conversely, prior research in the United States has suggested that the increase in calls during COVID was driven by households without a prior history of domestic violence (Leslie & Wilson, 2021). We also observe a significant group of women who started experiencing violence during this time, mainly psychological violence, opening new questions about factors associated with the increased risk of onset during confinement and the corresponding help-seeking responses. And while escalation in violence is uncommon in the data, prior research done in Chile showed increased shelter use following the implementation of lockdowns (Bhalotra et al., 2024), suggesting changes in intensity among those women at the highest risk. Unpacking the factors associated with continuity and escalation from those associated with onset is particularly relevant from both theoretical and policy perspectives, as the policies and services required would likely differ based on the level of prevention.

Finally, we found a marked drop in formal reporting of violence to authorities during the pandemic: the share of women who made formal complaints decreased by nearly half between 2020 and 2022. This decline suggests that help-seeking behavior may have been disrupted by pandemic conditions (see Bhalotra et al., 2024 for a similar argument) and underscores the limitations of relying on administrative records alone to assess changes in domestic violence during crises. One of the main challenges in studying crime during COVID-19 is separating changes in actual offending from changes in reporting behavior, as both may shift under policies such as mobility restrictions (Larroulet et al., 2025; Peterman & O'Donnell, 2020; Stickle & Felson, 2020). Survey data, by contrast, allow us to link changes in victimization to changes in reporting and to analyze how the pandemic affected both. This finding reinforces the value of victimization surveys in contexts where access to formal protection mechanisms may be limited, disrupted, or unevenly distributed, and it

motivates further research on the barriers women face when seeking help during periods of crisis and how institutions can adapt to maintain accessibility and responsiveness (García et al., 2022; Halford & Smith, 2022).

Limitations

The questions raised above outline several directions for future research. Some methodological limitations of the current work should be noted. First, conducting fieldwork during a public health emergency created barriers to participation, contributing to relatively high attrition. Although attrition was not associated with prior victimization, it may still limit generalizability if women facing the greatest constraints were less able to participate. Relatedly, while self-reports are essential for capturing violence that does not reach formal institutions—and especially relevant given the sharp decline in formal reporting documented above—general population surveys may not fully capture the most extreme cases of violence, due, in part, to fear of retaliation (Johnson, 1995; Johnson et al., 2014). As a result, escalation may be underrepresented in our panel.

Finally, because these data were collected under pandemic-specific conditions, it remains unclear whether the transition patterns observed here—such as shifts toward or away from psychological violence and movements into poly-victimization—reflect general IPV dynamics or crisis-specific mechanisms. Assessing their generalizability to non-emergency contexts is therefore an important next step.

Implications

While our study does not aim to evaluate interventions, the dynamic patterns documented here have implications for prevention and policy. The observed transitions—from violence to non-violence and vice versa—underscore that IPV is not static and requires multi-level (i.e., primary, secondary, tertiary) approaches adaptable to households whose risks evolve over time. These findings suggest that prevention efforts should be flexible enough to address both onset and escalation risks, as well as support recovery for those exiting violent relationships.

Furthermore, the sharp decline in formal reporting highlights the need to design crisis-resilient reporting and protection systems, ensuring accessibility during emergencies when standard pathways are disrupted. Strengthening remote, community-based, and mobile protection mechanisms may help reduce barriers during periods of confinement or restricted mobility. Interventions during crises should also combine

immediate safety measures with long-term support that accounts for changes in household composition, economic stress, and access to services (Weeks et al., 2024).

This study also carries implications for data collection on sensitive topics such as IPV, particularly under emergency conditions. We adapted the survey to COVID-19 and prioritized in-person interviews despite the difficulties. These decisions reflected the need to balance methodological consistency with ethical considerations, including the risks of administering IPV surveys in settings where perpetrators might be present (see Jessell et al., 2022 for a similar discussion). Strict social distancing protocols, shifting quarantines, and heightened emotional and economic stress posed real barriers to participation and contributed to a relatively high level of attrition. However, our analyses indicate that attrition was not systematically associated with prior victimization, which supports the validity of the decisions made.

Conclusions

We hope that the availability of this dataset expands the geographical scope of IPV research to the Latin American context. Although studies in the region increasingly highlight risk factors similar to those documented internationally—such as barriers to formal support (Carney, 2024), partners' heavy drinking and childhood exposure to violence (Castro et al., 2017), and unemployment (Bhalotra et al., 2021)—cultural and structural factors particular to Latin America also shape women's experiences (e.g., Restrepo Henao et al., 2022). By providing high-quality, longitudinal data, this project can support more contextualized analyses of the development and complexities of intimate partner violence in the Global South.

References

Arenas-Arroyo, E., Fernandez-Kranz, D., & Nollenberger, N. (2021). Intimate partner violence under forced cohabitation and economic stress: Evidence from the COVID-19 pandemic. *Journal of Public Economics, 194*, 104350. https://doi.org/10.1016/j.jpubeco.2020.104350

Axinn, W. G., Pearce, L. D., & Ghimire, D. (1999). Innovations in life history calendar applications. *Social Science Research, 28*(3), 243–264. https://doi.org/10.1006/ssre.1998.0641

Banco Mundial. (2021). *Hoja de ruta para el diseño de una plataforma integrada de gestión de casos de violencia contra las mujeres para el Estado chileno*. Banco Internacional de Reconstrucción y Fomento/Banco Mundial.

Bassuk, E., Dawson, R., & Huntington, N. (2006). Intimate partner violence in extremely poor women: Longitudinal patterns and risk markers. *Journal of Family Violence, 21*, 387–399. https://doi.org/10.1007/s10896-006-9035-1

Bhalotra, S., Britto, D. G., Pinotti, P., & Sampaio, B. (2021). *Job displacement, unemployment benefits and domestic violence* (IZA Discussion Paper).

Bhalotra, S., Clarke, D., Brito, E., Larroulet, P., Pino, F., (2024). Encuesta nacional de experiencias familiares y laborales durante la crisis de COVID-19, https://doi.org/10.7910/DVN/U8TIXK, Harvard Dataverse.

Bhalotra, S., Brito, E., Clarke, D., Larroulet, P., & Pino, F. (2026). Dynamic impacts of lockdown on domestic violence: Evidence from multiple policy shifts in Chile. *Review of Economics and Statistics*, *108*(2), 344-354 . https://doi.org/10.1162/rest_a_01412

Blofield, M., Knaul, F. M., Calderón-Anyosa, R., Peterman, A., Martinez Franzoni, J., O'Donnell, M., & Bustreo, F. (2022). A diagonal and social protection plus approach to meet the challenges of the COVID-19 syndemic: Cash transfers and intimate partner violence interventions in Latin America. *The Lancet Global Health, 10*(2), e148–e153. https://doi.org/10.1016/S2214-109X(21)00444-7

Bourgault, S., Peterman, A., & O'Donnell, M. (2021). *Violence against women and children during COVID-19—One year on and 100 papers in.* Center for Global Development.

Bott, S., Guedes, A., Ruiz-Celis, A. P., & Adams, J. M. (2021). La violencia por parte de la pareja íntima en las Américas: Una revisión sistemática y reanálisis de las estimaciones nacionales de prevalencia. *Revista Panamericana de Salud Pública.* https://doi.org/10.26633/RPSP.2021.34

Breiding, M. J., et al. (2014). *Intimate partner violence in the United States—2010.* National Center for Injury Prevention and Control, Centers for Disease Control and Prevention.

Brown, S. M., Doom, J. R., Lechuga-Peña, S., Watamura, S. E., & Koppels, T. (2020). Stress and parenting during the global COVID-19 pandemic. *Child Abuse & Neglect, 110*, Article 104699. https://doi.org/10.1016/j.chiabu.2020.104699

Bullinger, L. R., Carr, J. B., & Packham, A. (2021). COVID-19 and crime: Effects of stay-at-home orders on domestic violence. *American Journal of Health Economics, 7*(3), 249–280.

Carney, J. R. (2024). A systematic review of barriers to formal supports for women who have experienced intimate partner violence in Spanish-speaking countries in Latin America. *Trauma, Violence, & Abuse, 25*(1), 526–541. https://doi.org/10.1177/15248380231156196

Castro, R. J., Cerellino, L. P., & Rivera, R. (2017). Risk factors of violence against women in Peru. *Journal of Family Violence, 32*(8), 807–815. https://doi.org/10.1007/s10896-017-9929-0

Erten, B., Keskin, P., & Prina, S. (2022, May). Social distancing, stimulus payments, and domestic violence: Evidence from the US during COVID-19. In *AEA Papers and Proceedings* (Vol. 112, pp. 262–266).

Franceschet, S. (2010). Explaining domestic violence policy outcomes in Chile and Argentina. *Latin American Politics and Society, 52*(3), 1–29.

Garcia, R., Henderson, C., Randell, K., Villaveces, A., Katz, A., Abioye, F., DeGue, S., Premo, K., Miller-Wallfish, S., Chang, J. C., Miller, E., & Ragavan, M. I. (2022). The impact of the COVID-19 pandemic on intimate partner violence advocates and agencies. *Journal of Family Violence, 37*(6), 893–906.

Garcia-Moreno, C., Jansen, H. A. F. M., Ellsberg, M., Heise, L., & Watts, C. H. (2006). Prevalence of intimate partner violence: Findings from the WHO multi-country study on women's health and domestic violence. *The Lancet, 368*(9543), 1260–1269. https://doi.org/10.1016/S0140-6736(06)69523-8

Gibbons, M. A., Murphy, T. E., & Rossi, M. A. (2021). Confinement and intimate partner violence. *Kyklos, 74*(3), 349–361. https://doi.org/10.1111/kykl.12275

Giordano, P. C., Johnson, W. L., Manning, W. D., Longmore, M. A., & Minter, M. D. (2015). Intimate partner violence in young adulthood: Narratives of persistence and desistance. *Criminology, 53*(3), 330–365. https://doi.org/10.1111/1745-9125.12073

Goodlin, W. E., & Dunn, C. S. (2010). Three patterns of domestic violence in households: Single victimization, repeat victimization, and co-occurring victimization. *Journal of Family Violence, 25*, 107–122.

Halford, E., & Smith, J. (2022). Operation Provide: A multi-agency response to increasing police engagement in cases of intimate partner violence during the COVID-19 pandemic. *Police Practice and Research, 23*(5), 600–613. https://doi.org/10.1080/15614263.2022.2033621

Haslag, P., & Weagley, D. (2024). From L.A. to Boise: How migration has changed during the COVID-19 pandemic. *Journal of Financial and Quantitative Analysis, 59*(5), 2068–2098. https://doi.org/10.1017/S002210902300073X

Hayes, B. E. (2018). Benefits and challenges of using life history calendars to research intimate partner violence. *Journal of Family Violence, 33*, 227–238.

Heise, L. L., & Kotsadam, A. (2015). Cross-national and multilevel correlates of partner violence: An analysis of data from population-based surveys. *The Lancet Global Health, 3*(6), e332–e340. https://doi.org/10.1016/S2214-109X(15)00013-3

Hernández, W., Cozzubo, A., Aguilar, J. C., Agüero, J. M., & Mendoza, J. (2024). Violence in pandemic times: The dynamic relationship between COVID-19 and intimate partner violence. *Review of Development Economics, 28*(2), 432–461. https://doi.org/10.1111/rode.13059

Jessell, L., Zaidi, I., Dominguez-Gomez, L., & Harocopos, A. (2022). Collecting data during the COVID-19 pandemic: Lessons from an in-person survey of people who use opioids. *Public Health Reports, 137*(2), 272–277. https://doi.org/10.1177/00333549211063473

Johnson, M. P. (1995). Patriarchal terrorism and common couple violence: Two forms of violence against women. *Journal of Marriage and Family, 57*(2), 283–294.

Johnson, M. P., Leone, J. M., & Xu, Y. (2014). Intimate terrorism and situational couple violence in general surveys: Ex-spouses required. *Violence Against Women, 20*(2), 186–207.

Kim, B., & Royle, M. (2024). Domestic violence in the context of the COVID-19 pandemic: A synthesis of systematic reviews. *Trauma, Violence, & Abuse, 25*(1), 476–493.

Larroulet, P., Pailañir, D., Quintana, D., & Clarke, D. (2025). Schools as safety nets. *Journal of Human Resources.* Advance online publication. https://doi.org/10.3368/jhr.0523-12924R1

Lei, L., & Liu, X. (2022). The COVID-19 pandemic and residential mobility intentions in the United States: Evidence from Google Trends data. *Population, Space and Place, 28*, e2581. https://doi.org/10.1002/psp.2581

Leslie, E., & Wilson, R. (2020). Sheltering in place and domestic violence: Evidence from calls for service during COVID-19. *Journal of Public Economics, 189*, Article 104241. https://doi.org/10.1016/j.jpubeco.2020.104241

Miller, A. R., Segal, C., & Spencer, M. K. (2024). Effects of the COVID-19 pandemic on domestic violence in Los Angeles. *Economica, 91*(361), 163–187. https://doi.org/10.1111/ecca.12493

McNeil, A., Hicks, L., Yalcinoz-Ucan, B., & Browne, D. T. (2023). Prevalence & correlates of intimate partner violence during COVID-19: A rapid review. *Journal of Family Violence, 38*(2), 241–261. https://doi.org/10.1007/s10896-022-00386-6

Mena, G. E., Martinez, P. P., Mahmud, A. S., Marquet, P. A., Buckee, C. O., & Santillana, M. (2021). Socioeconomic status determines COVID-19 incidence and related mortality in Santiago, Chile. *Science, 372*(6545), eabg5298.

Ogle, C. M., Harmon-Darrow, C., Fedina, L., Nichols, D., Mulford, C. F., & Backes, B. L. (2024). Operational definitions of poly-victimization: A scoping review. *Trauma, Violence, & Abuse, 25*(4), 3360–3376. https://doi.org/10.1177/15248380241246522

Perez-Vincent, S. M., & Carreras, E. (2022). Domestic violence reporting during the COVID-19 pandemic: Evidence from Latin America. *Review of Economics of the Household, 20*(3), 799–830. https://doi.org/10.1007/s11150-022-09607-9

Peterman, A., & O'Donnell, M. (2020). *COVID-19 and violence against women and children: A second research round up.* Center for Global Development.

Piquero, A. R., Jennings, W., Jemison, E., Kaukinen, C., & Knaul, F. (2021). Domestic violence during COVID-19. *National Commission on COVID-19 and Criminal Justice,* February.

Ravi, K. E., Cronley, C., Lawler, A., & Held, M. L. (2024). Presenting methodological resilience for conducting research with vulnerable populations during current and future pandemics: A case study

with IPV shelters and survivors in the United States. *Journal of Family Violence, 39*(5), 827–834. https://doi.org/10.1007/s10896-023-00499-6

Ravindran, S., & Shah, M. (2023). Unintended consequences of lockdowns, COVID-19 and the shadow pandemic in India. *Nature Human Behaviour, 7*(3), 323–331. https://doi.org/10.1038/s41562-022-01572-0

Restrepo Henao, A., Montoya Gómez, N. E., & Zuluaga Bedoya, L. V. (2022). Typologies of intimate partner violence against women in five Latin-American countries: A latent class analysis. *International Journal of Public Health, 67*. https://doi.org/10.3389/ijph.2022.1604000

Richards, T. N., Nix, J., Mourtgos, S. M., & Adams, I. T. (2021). Comparing 911 and emergency hotline calls for domestic violence in seven cities: What happened when people started staying home due to COVID-19? *Criminology & Public Policy, 20*(3), 573–591.

Roberts, J., & Horney, J. (2009). The life event calendar method in criminological research. In *Handbook of Quantitative Criminology* (pp. 289–312). Springer. https://doi.org/10.1007/978-0-387-77650-7_15

Saavedra, T., Contreras, M., & Inchauste, M. G. (2022). *Intimate partner violence against women: Prevalence, formal reporting, and risk factors in Chile* (Policy Research Working Paper No. 10016). World Bank Group. https://doi.org/10.2139/ssrn.4554007

Stickle, B., & Felson, M. (2020). Crime rates in a pandemic: The largest criminological experiment in history. *American Journal of Criminal Justice, 45*(4), 525–536. https://doi.org/10.1007/s12103-020-09546-0

Treves-Kagan, S., Peterman, A., Gottfredson, N. C., Villaveces, A., Moracco, K. E., & Maman, S. (2022). Equality in the home and in the community: A multilevel longitudinal analysis of intimate partner violence on the Ecuadorian-Colombian border. *Journal of Family Violence, 37*, 1–14. https://doi.org/10.1007/s10896-020-00218-5

UNODC, United Nations Office on Drugs and Crime. (2024). *Femicides in 2023: Global estimates of intimate partner/family member femicides.*

Voce, I., & Boxall, H. (2018). Who reports domestic violence to police? A review of the evidence. *Trends & Issues in Crime and Criminal Justice, (559)*, 1–16.

Weeks, L. E., Stilwell, C., Rothfus, M., Weeks, A. J., Macdonald, M., Jackson, L. A., ... & Daclan, A. (2024). A review of intimate partner violence interventions relevant to women during the COVID-19 pandemic. *Violence Against Women, 30*(3–4), 981–1021.

White, S. J., Sin, J., Sweeney, A., Salisbury, T., Wahlich, C., Montesinos Guevara, C. M., Gillard, S., Brett, E., Allwright, L., Iqbal, N., Khan, A., Perot, C., Marks, J., & Mantovani, N. (2024). Global prevalence and mental health outcomes of intimate partner violence among women: A systematic review and meta-analysis. *Trauma, Violence, & Abuse, 25*(1), 494–511. https://doi.org/10.1177/15248380231155529